



CASE PRESENTATION

GASTRIC OUTLET OBSTRUCTION

Dhivakhar S final year
Unit 7

DEMOGRAPHIC DETAILS

NAME : MR. ARUN KUMAR

AGE : 65YRS SEX : MALE

OCCUPATION : MANUAL LABOUR

ADDRESS : INDRA NAGAR ,PORUR

SOCIOECONOMIC STATUS : LOWER MIDDLE CLASS



CHIEF COMPLAINTS

- pain in abdomen since 3 months
- Vomiting after food since 2 months



HISTORY OF PRESENTING ILLNESS

- Patient was apparently normal 3 months ago after which Patient complaints of pain in the upper abdomen since 3 months
 - Insidious in onset
 - Initially it was intermittent in nature ;non radiating ,non periodic
 - Aggravated by food ingestion & relieved by antacids
 - It did not affect his daily activities and there was no episodes of nausea and vomiting ,Water brash or heart burn sensation
- But since past 1 month it has gradually increased in severity and is now dull aching type, continuously present



- Vomiting since 2 months
 - Spontaneously vomiting
 - Initially vomiting was after 1-2hrs of taking food later vomiting occurs immediately after food [within few mins]
 - non-projectile ,non-bilious, blood stained, contain undigested food particles
 - tolerates fluid
- Malena since 10days;reduced frequency of bowel movements ,once in 3-4 days ;No h/o consumption of iron supplements



- H/o early satiety
- H/o loss of appetite since 2 months
- H/o loss of weight [before 3 months-60kg;now-52kg]
- H/o generalized weakness & easy fatiguability since 1 month which affected his daily activities
- H/o ball rolling movement
- No h/o discolouration of urine, eyes & skin
- No h/o breathlessness/hemoptysis
- No h/o bone pain
- No h/o evening rise of temperature
- No h/o fever
- No h/o skin pigmentation



PAST HISTORY

- No h/o similar complaints in the past
- No h/o previous hospitalization, blood transfusion and surgery
- Not a known case of diabetes mellitus, hypertension, epilepsy, tb, asthma & ischemic heart disease
- No h/o peptic ulcer disease as evidence by absence of periodicity pain
- No h/o any drug intake [for pud/nsaids]
- No h/o allergy to any food /drug



FAMILY HISTORY

- No h/o any malignancy or malignancy related death in the family



PERSONAL HISTORY

- Consumes mixed diet
- Patient is a chronic alcoholic ; consumes 90ml brandy/day x 20yrs
- Chronic smoker;10 cigarettes/day x 20yrs
- No h/o other addictive habits
- Bowel habits : loss of appetite and reduced frequency of bowel
- Bladder habits : normal & regular



SUMMARY

A 65 yr old male presented with dull aching pain in the upper abdomen, vomiting immediately after food which is non –projectile, non- bilious with loss of weight and loss of appetite with no h/o of periodicity pain. patient is a chronic alcoholic and smoker. These features are suggestive of gastric outlet obstruction probably due to malignant cause.



GENERAL EXAMINATION

- Patient is conscious ,cooperative and oriented to time, place and person.
- Moderately built and poorly nourished
- Pallor present
- No icterus
- No clubbing
- No cyanosis
- No pedal edema
- No generalized lymphadenopathy



VITALS

- BP:120/70mmHg
- RR:18/min
- PR:85BPM
- TEMPERATURE:AFEBRILE



EXAMINATION OF ABDOMEN

INSPECTION

- Shape of abdomen: normal
- Umbilicus: in midline and inverted, no nodules or discoloration seen around umbilicus
- All quadrants move equally with respiration
- No visible mass
- No visible pulsation
- Visible gastric peristalsis present
- No dilation of veins
- No scars/sinuses
- Hernial orifice free
- External genitalia normal



PALPATION

- No warmth and tenderness over abdomen
- A vague mass is palpable in the epigastric region extending into left hypochondrium with irregular borders of size 3cmx3cm,hard in consistency,non tender,moves with respiration.
- Upper border not palpable
- Mass becomes less prominent on head raising test
- No intrinsic mobility
- Non pulsatile
- Liver and spleen not palpable
- Hernial orifice:intact
- No palpable lymph nodes



PERCUSSION

- Dull note over the mass
- Resonant note present over rest of abdomen
- No free fluid



AUSCULTATION

- Bowel sound is heard
- Succussion splash heard



PER RECTAL EXAMINATION

- Inspection: normal
- Digital palpation:-
 - Tone of sphincter: normal
 - Within the lumen: pellets of stool present
 - Rectal wall: mucosa is smooth with no deposits
 - Outside the wall: no palpable mass
 - Gloved finger stained with dark stools



OTHER SYSTEM EXAMINATION

- RS: normal vesicular breath sounds
- CVS: S1,S2 heard .No added sounds
- CNS: no focal neurological deficits



PROVISIONAL DIAGNOSIS

Gastric outlet obstruction with features of carcinoma of stomach



INVESTIGATION

- **ROUTINE INVESTIGATION**
- COMPLETE HEMOGRAM
- BLOOD SUGAR, UREA
- SERUM CREATININE
- SERUM ELECTROLYTE
- BLOOD GROUPING AND TYPING
- X-RAY CHEST AND ECG



SPECIFIC INVESTIGATIONS

- USG ABDOMEN
- CECT
- UPPER GASTROINTESTINAL ENDOSCOPY
- BIOPSY
- BARIUM MEAL
- CEA [CARCINOEMBRYONIC ANTIGEN]
- DIAGNOSTIC LAPROSCOPY



TREATMENT

- Decided based on investigation, lymph node status, operability of tumor.
- If operable tumor, tumor at the lever of pylorus- sub total gastrectomy with bilroth II anastomosis D2 dissection with adjuvant chemotherapy



THANK YOU

